SACTCHECK · REGIMEN-SPECIFIC SACT CONSENT

PROTOTYPE · GOVERNANCE REVIEW REQUIRED

Carboplatin every 3 weeks with head and neck radiotherapy

Locally advanced stage III–IV head and neck squamous cell carcinoma when cisplatin is not appropriate. · NCCP 00261 · version 7 · 21-day cycle

Patient name	DOB	Hospital no.	Date

Treatment context
Carboplatin is given with radiotherapy to treat locally advanced head and neck cancer when cisplatin is unsuitable. Your team explains the intended chance of cure or long-term control for your diagnosis.

Schedule
Carboplatin is given through a vein over about 30 minutes on Day 1 every 21 days. The head and neck prescription uses AUC 5, a target used with your checked kidney function to calculate the dose. The whole visit includes safety checks, anti-sickness medicines, preparation and observation. Three carboplatin cycles are planned with concurrent radiotherapy. Radiation has a separate appointment calendar; the team coordinates any interruptions or changes.

1. TREATMENT DISCUSSION

- ☐ Purpose, expected benefit and alternatives explained
- ☐ Indication, medicines, sequence and duration confirmed
- ☐ Blood tests and any individually indicated heart checks discussed
- ☐ Support medicines and phase changes explained
- ☐ Fertility, contraception and breastfeeding discussed where relevant
- ☐ Common, serious and lasting drug-specific risks discussed below
- ☐ Urgent symptoms and local contact details confirmed

2. REGIMEN / AGENT-SPECIFIC MATERIAL RISKS

Carboplatin
PLATINUM CHEMOTHERAPY

- ☐ Low blood counts, infection and bleeding
- ☐ Nausea, vomiting and fatigue
- ☐ Kidney, hearing and nerve effects
- ☐ Infusion allergy, particularly with repeat platinum exposure
- ☐ Fertility effects and harm to an unborn baby

Radiotherapy
LOCAL TREATMENT

- ☐ Sore mouth and throat
- ☐ Dry mouth, taste and saliva changes
- ☐ Swallowing or voice problems
- ☐ Skin reaction in the treated area
- ☐ Jaw stiffness and dental problems
- ☐ Some swallowing, saliva or tissue effects can be lasting

Discuss your diagnosis, intent, alternatives, expected benefit and uncertainty, treatment burden, serious or lasting harms, fertility and pregnancy precautions and your priorities. Include blood-count suppression, serious infection and bleeding, nausea/vomiting, allergy, renal dose calculation, hearing and nerve effects, and the separate radiotherapy consent covering mouth, saliva, skin, dental, jaw and swallowing effects. Confirm actual urgent contacts and use teach-back. This draft does not authorise treatment.

Individual aim / benefit, alternatives, material risks and questions:

3. CONSENT CONFIRMATION

- ☐ Expected benefit, important risks and reasonable alternatives discussed
- ☐ Patient questions invited and answered; patient-specific material risks / priorities considered
- ☐ Emergency contact / toxicity advice and written information offered where appropriate
- ☐ Patient understands consent is voluntary and may be withdrawn

Patient / person giving consent _____ Date _____

Clinician obtaining consent _____ Date _____

Prototype only. This one-page form is a regimen-specific consent aid and requires formal governance, consultant and oncology-pharmacy review before clinical use. The current NCCP regimen, local/national consent policy, current medicine information, patient-specific material risks and clinical judgement remain authoritative.
Base regimen: NCCP 00261 v7.

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